M32 Paracetamol (Acetaminophen)

Scope of Practice	EMR **PO** administration EMT **IV/PR** administration
Classification	• Non-opioid analgesic and antipyretic
Usual Presentation	• Oral Tablets: 500mg/1 tablet • Rectal Suppositories: 125mg and 250mg/suppository. • Bottle for IV administration: 1000mg/100ml
Mechanism of Action	• Paracetamol works primarily by inhibiting prostaglandin synthesis in the central nervous system.
Indications	• Moderate to Severe Pain • Fever >38°C
Contraindications	• Known allergy • If patient has already taken paracetamol within the last 4 hours, or if the maximum daily dose has already been reached.
Precautions	• Nil
Side Effects	• Rapid administration of IV paracetamol may cause hypotension. • Pruritus • Nausea • Vomiting
Adult Dose & Administration	• IV/PO: 1000mg (maximum 4g in 24 hours) • IV doses should be administered slowly over 10 minutes to avoid toxicity.
Pediatric Dose & Administration	• PR: <5 years: 125mg | >5 years: 250mg • IV: 10mg/kg • IV doses should be administered slowly over 10-15 minutes to avoid toxicity.
Use in Pregnancy and Lactation	• Pregnancy: Safety has not been established. However, paracetamol is generally considered safe to use in pregnancy and may be administered when clinically indicated. • Lactation: Safety has not been established. However, paracetamol is generally considered safe to use in lactation and may be administered when clinically indicated.
Additional Information	• Caution should be utilized when administering IV paracetamol to pediatrics without a syringe driver, infusion pump or micro-drip set due to the inability to accurately administer the dose. • IV administration to pediatrics <33kg is generally not recommended due to the risk of toxicity.